A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

30 April 2026

Dr Chung Ping Ben Ooi
Medical Hub @RMIT

Dear Dr Ooi,

Re: Mr Parker M Kenworthy
DOB: 26/09/2005 Phone: 0423 838 536
Address: 0530-7, 50 La Trobe Street, MELBOURNE, VIC, 3000
Email: parker.kenworthy2@gmail.com
Medicare: 6111654131 / 1

Summary

- **Severe gastro-oesophageal reflux symptoms with associated dysphagia and alternating bowel habit.**
- **For gastroscopy and colonoscopy to exclude eosinophilic oesophagitis and inflammatory bowel disease.**
- **No action required by you.**

I had the pleasure of conducting an in-person consultation with Parker Kenworthy, thank you for referring him.

Symptoms and history of presenting complaint

Parker presents with a long-standing history of severe gastro-oesophageal reflux symptoms, dating back to approximately 12 years of age. The symptoms are particularly debilitating at night, characterized by acid regurgitation into the posterior pharynx, which frequently results in a choking sensation and necessitates sitting upright to regain breath. Interestingly, Parker does not report typical retrosternal classic heartburn but describes frequent effortless regurgitation of both gastric contents and undigested food.

Furthermore, there is a clear history of intermittent dysphagia. Parker reports that solid boluses, such as crusty bread and meat, as well as larger tablets, occasionally feel lodged in the mid-oesophageal region, requiring water flushes to facilitate passage.

Regarding bowel habit, Parker describes a volatile pattern of alternating constipation and near-liquid diarrhoea. These symptoms are associated with significant abdominal pain and a persistent sense of incomplete evacuation. There is a notable history of two hospitalisations for suspected gastroenteritis at ages 5 and 16; the latter was particularly severe.

Diet

Parker has attempted a low-FODMAP diet with only marginal symptomatic improvement. Current dietary triggers include coffee, carbonated beverages, and certain fruits, while there is a suspected intolerance to fructans rather than gluten. Meals are often irregular due to university and work commitments, with frequent skipping of breakfast.

Past medical history and medications

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
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Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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Parker's medical history is significant for ADHD and a history of gender-affirming top surgery at age 18. Previous hospitalisations occurred for the aforementioned episodes of severe gastrointestinal distress. Current medications include pantoprazole 40mg daily (recently increased from 20mg), Vyvanse for ADHD, and previous Reandron injections. There are no known drug allergies.

Social history, family history

Parker is currently in the third year of a Bachelor of Communications at RMIT and operates a private tutoring business. Alcohol consumption is modest, limited to wine once every fortnight, and there is no history of smoking. There is a strong maternal family history of gastrointestinal pathology, including Barrett's oesophagus in a maternal grandfather and aunt, and a hiatus hernia in Parker's mother.

Examination On physical examination, there was evidence of mild tenderness upon palpation of the epigastrium. The remainder of the abdominal examination was unremarkable, with no palpable masses or signs of peritonism.

Impression and plan

The clinical picture is suggestive of oesophageal dysmotility/reflux with a differential of EoE, though the degree of dysphagia in a young patient necessitates the exclusion of eosinophilic oesophagitis (EoE) or a primary oesophageal motility disorder. The family history of Barrett's oesophagus also warrants a thorough endoscopic evaluation of the distal oesophageal mucosa. Parker's bowel symptoms, while potentially functional, require investigation to rule out organic causes such as Crohn's disease or microscopic colitis.

I have also discussed the concept of visceral hypersensitivity. It is likely that an amplified neural response is contributing to the severity of both the reflux and the lower gastrointestinal symptoms.

Our plan is as follows:

- **Combined gastroscopy and colonoscopy:** This will be performed under sedation to assess for structural abnormalities, including hiatus hernia, and to obtain biopsies for EoE, coeliac disease, and colonic pathology.
- **Medication management:** I have provided a prescription for pantoprazole 40mg to be taken 30 minutes prior to food.
- **Review:** We will convene to discuss the histopathology results and consider the role of neuromodulators if symptoms suggest a significant component of visceral hypersensitivity.

Thank you again for referring this patient.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

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